Anxiety disorders

Goal

To provide the participants with knowledge, skills and positive attitude toward anxiety disorders.

Objectives:

At the end of this lecture, the participants will be able to:

- Define fear, stress, anxiety and anxiety disorder.
- Differentiate between anxiety and anxiety disorders.
- Identify etiology of anxiety disorders.
- Clarify the types of anxiety disorders and signs & symptoms of anxiety.
- Apply line of treatment and nursing management for anxiety.
- Gain positive attitude toward anxiety disorders.

Outlines:

1. Introduction.
2. Definition:
   - ✓ Fear.
   - ✓ Stress.
   - ✓ Anxiety.
   - ✓ Anxiety disorders.
3. Difference between normal and abnormal anxiety.
4. Etiology of anxiety disorders.
5. Types of anxiety.
6. Line of treatment
7. Nursing management.

Introduction:

- Experiencing occasional anxiety is a normal part of life. However, people with anxiety disorders frequently have intense, excessive and persistent worry and fear about everyday situations. Often, anxiety disorders involve repeated episodes of sudden feelings of intense anxiety and fear or terror that reach a peak within minutes in some cases (panic attack).

Definitions:

Item	Definition
Fear	Is a natural emotion .it alerts us to the presence of dangers whether they are real dangers or imagined.
Stress	Is pressure or tension that puts us into fight or flight mode from an external trigger.
Anxiety	Is a feeling of worry, nervousness or fear with an uncertain outcome that is internal and not dependent on an external trigger
Anxiety disorders	Are a group of mental illnesses that are characterized by excessive nervousness, fear, apprehension, and worry that greatly impair the ability to function normally , It led to changes in behavior, sleep, eating, or mood.

Difference between normal and abnormal anxiety:

	Normal anxiety	Pathological anxiety
Duration	Short-lived; resolves once the stressor is removed	Lasts for weeks, months, or years.
Trigger	Specific stressors (e.g., exams, job interviews, public speaking	Can occur without an identifiable cause or persist after the stressor is gone.
Impact on daily life	Does not significantly interfere with work, relationships, or daily activities.	Causes significant impairment in personal, social, or professional life.
Need to treatment	Usually does not require professional treatment.	Often requires therapy, medication, or both.

Causes of Anxiety Disorders:

1. Genetic factors.
2. Brain chemicals as Serotonin, GABA and Noradrenaline.
3. Environmental stress.
4. Substance abuse.
5. Mental health conditions.
6. Medical conditions.
7. Childhood experience.
9. Social psychological factors.

Types of anxiety disorders:

1. Generalized Anxiety Disorder. (GAD)
2. Panic Disorders.
3. Social Anxiety Disorders (social phobia) .
4. Phobias.
5. Agoraphobia.
6. Separation anxiety Disorders.
7. Selective mutism.
8. Medication/substance induced anxiety disorders.

1. Generalized Anxiety Disorder (GAD)

- This disorder is characterized by persistent, unrealistic, and excessive anxiety and worry, which have occurred more days at least 6 months.
- Symptoms are not situational or limited to certain events.

DSM-5 criteria:

1. Restlessness.
2. Feeling keyed up
3. Easily fatigued
4. Difficulty in concentrating
5. Irritability
6. Muscle tension
7. Sleep disturbances
8. eating disturbances
9. Sweating
10. Nausea
11. Diarrhea
12. Accelerated heart rate
13. Shortness of breath

2. Panic Disorders

- **Panic disorder (PD)** is defined as recurrent, unexpected panic attacks followed by at least 1 month of persistent concern about having another attack, worry about the consequences of panic attacks, and a change in behavior as a result of the attacks.
- Mimics a heart attack or feeling that death is imminent
- Can occur at any time, even during sleep

DSM-5 criteria:

1. Palpitations, increased heart rate
2. Sweating
3. Hyperventilation.
4. Chest pain or discomfort
5. Trembling or shaking
6. Persistent worry about having future attack.
7. sense of impending doom

3- Agoraphobia.

- Significant anxiety or fear to place or situation with concurrent fear that one cannot escape the place.

DSM-5 criteria:.

- **To meet criteria for agoraphobia, two of the following situations need to cause the person significant anxiety or fear**

(a) Using public transportation.

(b) Being in open spaces.

(c) Being in enclosed spaces.

(d) Being in a crowd.

(e) Being outside one's home alone.

4- Specific phobias:

- Excessive fear or anxiety about specific object or situation (flying, heights, and animals, receiving syringes or seeing blood.
- The avoidant behavior or fear and anxiety lasts for at least 6 months and results in work and social impairment.
- Systematic desensitization (graduated expose therapy) and behavioural therapy.

Types of Specific phobias

1. **Zoo phobia** (For ex., dread of spiders or snakes).
2. **Social phobia:** dread of public humiliation, as in fear of public speaking, performing, or eating in public.
3. **Acrophobia**: fear of high places.
4. **Agoraphobia**: fear of open places.
5. **Claustrophobia**: fear of closed places.
6. **Algophobia**: fear of pain.
7. **Erythrophobia**: fear of red (refers to a fear of blushing).
8. **Xenophobia**: fear of strangers

5- Social anxiety disorder (Social phobia):

- **Social phobia** is an intense fear or anxiety in social situations when an individual believes he or she is being negatively evaluated by others.
- To distinguish social anxiety from a transient social fear, the disturbance usually spans **at least 6 months.**
- The disturbance significantly interferes with social functioning or occupational functioning.

DSM-5 Diagnostic Guidelines

1. **Feeling anxious in social situation.**
2. **Fear of being judged, criticized or humiliated.**
3. **Avoiding of social situations and activities.**
4. **Fear of being watched or evaluated by others.**
5. **Physical symptoms as (tachycardia sweating and tremors)**.

6- Separation anxiety disorder:

- Intense fear or anxiety due to separation from attachment figure.
- It can affect children and adults as well.

DSM-5 criteria:

1. Excessive worry of losing attachment figures or being alone.
2. Physical symptoms as (nausea, headache, gastric pain).
3. Behavioral symptoms as (refuse to go to school, have temper tantrum and insist to be with the attachment figure)
4. symptoms must persist for a period of at least 4 weeks for children and adolescents younger than 18 years and persist for a period of 6 months or longer for adults.

7- Selective mutism

- Children with selective mutism demonstrate a lack of speech in social situations and do not initiate speech or respond with speech.
- Children with selective mutism will speak in their homes with immediate family members but tend not to speak with extended family or friends.
- They often refuse to speak at school, leading to academic problems.
- Shyness, anxiety, clinging behavior, withdrawal, and social isolation are common associated features.
- Children with selective mutism generally have normal language skills and do not have any communication disorder.

DSM-5 Diagnostic Guidelines

1. Failure to speak in social situations where there is an expectation for speaking despite the ability to speak
2. The failure to speak significantly interferes with education and/or occupational development.
3. The disturbance lasts at least 1 month.
4. The failure to speak is not due to a lack of language development or inability to use spoken language that would be required in a social situation.
5. Failure to speak is not related to a communication disorder.

Line of treatment:

A. Hospitalization:

1. In case of Suicidal ideation or homicidal hospitalization is required.
2. Person experience sever symptoms enough to impair their ability to function at work and home.
3. Person require high level of care.

B. Psychotherapy:

- **Cognitive Behavioral Therapy:** is the most widely used therapy for anxiety disorders. Research has found it to be effective in treating SAD, GAD, phobias, and panic disorders, among other conditions.

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C. Pharmacological:

Antidepressant agents are the drugs of choice in the treatment of anxiety disorders:

1. **Selective serotonin reuptake inhibitors:** The SSRIs are first-line agents for long-term management of anxiety disorders.
2. **Serotonin and norepinephrine reuptake inhibitors**
3. **BNZs in severe cases and during start dose.**

Nursing Management

- Assess the intensity of anxiety
- Determine the triggers for anxiety
- Assess how the patient responds to anxiety
- Administer medications to relieve anxiety
- Educate the patient about anxiety
- Encourage patient to develop support groups
- Encourage patient to seek mental health counseling
- Educate patient on self-care
- Provide means of support

CONT.,

- Interact with the patient in a calm and gentle manner
- Converse in simple language
- Allow the patient to talk about distressing emotions and feelings
- Assess patient for suicidal ideations
- Help strengthen patient’s problem-solving abilities
- Tell the patient to limit alcohol and caffeinated beverages
- Encourage patient to participate in social functions

Thank You

Any question?